Support airway, breathing and circulation
as per appropriate CPG

- Maintain Sp02 94-98%
- ETCO2 monitoring
- Gain IV access

Consider ALS Back-Up

Consider Requesting Police Assistance

Consider pre-oxygenation prior to naloxone administration

Naloxone IN
Adults: 4mg
initial dose.
Repeat after 3minutes if required.
Max dose 20mg

EMT

OR

Naloxone IV/IO/IM
Adults: 0.4mg
Repeat after 3 minutes if required

EMT

Naloxone IV/IO/IM
Pediatric: 0.01mg/kg
Repeat after 3 minutes if required

EMT

Patient Care Goals

1. Rapidly recognize and manage life-threatening effects of Opioid overdose
2. Support airway, breathing, and circulation while minimizing aspiration risk
3. Identify opioid toxicity and administer naloxone (Narcan)
4. Transport to definitive care for monitoring, or antidotal therapy
5. Prevent complications through appropriate monitoring and communication

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- Suspected or confirmed intentional or unintentional opioid overdose symptoms including:
  - Altered mental status
  - Respiratory depression
  - Cardiovascular instability
  - Seizure or coma
  - History or evidence of opioid ingestion

Exclusion Criteria

- Poisoning from industrial/environmental toxins (See General Toxicology Management CPG).
- Ingestion of non-toxic substances with no clinical symptoms
- Isolated alcohol intoxication (addressed in separate guideline)

Key Patient Assessment Points

1. **Scene Clues**
   - Pill bottles, syringes, patches, empty blister packs
   - Bystander report, suicide note, known substance use disorder
2. **Airway & Breathing**
   - Assess for snoring respirations, cyanosis, shallow breathing
   - Suspect opioid toxicity if RR <12 with pinpoint pupils
3. **Circulation**
   - Bradycardia or hypotension (opioids, beta-blockers)
   - Tachycardia and hypertension (amphetamines, cocaine)
4. **Neurological Status**
   - GCS <15, agitation, confusion, seizures, or unresponsiveness
   - Pupils: miosis (opioids), mydriasis (sympathomimetics or withdrawal)
5. **Clinical Indicators of Opioid Toxicity Assessment**
   - Respiratory rate <12
   - Pinpoint pupils
   - Depressed LOC
   - Evidence of opioid use or paraphernalia
6. **High-Risk Indicator Assessment**
   - Identify patients at higher risk of severe outcomes, respiratory arrest, or complications:
     - GCS <8
     - Respiratory rate <10 or apnea
     - SpO_2 <94% despite oxygen
     - Pinpoint pupils with poor respiratory effort
     - Suspected co-ingestants (e.g., alcohol, benzodiazepines, stimulants)
     - History of chronic opioid use or recent release from incarceration (high overdose risk due to loss of tolerance)
     - Pediatric ingestion; even a small dose of opioid can be fatal
     - Extended-release or long-acting opioid ingestion (risk of re-narcotization)
     - Suspected intentional overdose or suicide attempt
7. **SAMPLE History**

- S (Symptoms): What did the patient complain of before collapsing (e.g., drowsiness, vomiting, chest pain, confusion)?
- A (Allergies): Any known drug allergies, especially to naloxone or medications that may be involved
- M (Medications): Look for prescription opioids, sedatives, antidepressants, or illicit drug use; ask about patches, injections, or recent hospital discharge
- P (Past medical history): History of substance use disorder, mental health conditions, prior overdose, chronic pain
- L (Last oral intake): What and when did they last eat, drink, or take any medication or substance?
- E (Events): Timeline of the event — When was the patient last seen normal? Was there a witnessed ingestion or collapse? Any bystander CPR or naloxone given before EMS arrival?

8. **Determine Triage Category**
   - Determine Triage Category as per Patient Triage Categories CPG.
     - Red Priority: Respiratory depression, GCS <8
     - Yellow Priority: Stable vitals, moderate symptoms (e.g., drowsy, hypotensive)
     - Green Priority: Alert, no current symptoms, accidental ingestion

Patient Management & Interventions

1. **Consider ALS back-up**
   - Consider the requirement for ALS back-up as per Universal Care CPG.
   - ALS back-up should be routinely requested for:
     - GCS <8
     - Respiratory compromise
     - Seizures or significant cardiovascular effects
     - Suspected poly-drug ingestion
1. **Primary Survey**
   - Perform Primary Survey as per Universal Care CPG.
     - Provide BVM ventilation if RR <10 or SpO_2 <94%
     - Suction as needed to clear airway from vomitus
2. **Oxygen Therapy**
   - Administer high-flow O_2 for all patients with hypoxia or respiratory depression
   - Target SpO_2 ≥94%
3. **Opioid Overdose Management**
   - Administer Naloxone:
     - Indications:
       - Signs and Symptoms consistent with opioid overdose, and/
         - Altered Mental Status, or
         - Respiratory Depression, or
         - Cardiovascular compromise.
   - Naloxone Dosage and Administration

Naloxone (Narcan)		
Route	**Dose**	**Notes**
IN (Intranasal)	4 mg (single puff per nostril)	Repeat every 2–3 min if needed, max 8 mg
IM or IV	0.4mg	Titrate to respiratory effort, may repeat every 2–3 min, max 10 mg

   - If no response after 10 mg total → suspect alternative diagnosis
   - Maintain ventilation support — naloxone has shorter half-life than some opioids
   - Be prepared for agitation or withdrawal after reversal
4. **Patient Reassessment**
   - Frequently Reassess the patient as per Universal Care CPG.
   - Specific Patient Reassessment Points:

- Respiratory rate and effort
- SpO_2
- GCS and pupil size
- Heart rate and blood pressure

5. **Transport Considerations**
   - Transport all opioid overdose patients, even if they initially respond to naloxone
   - Provide early notification to receiving facility if airway or cardiovascular support is required
   - Notify dispatch and police in suspicion of drug overdose before transport

Patient Safety Considerations

- Maintain airway protection at all times
- Do not rely on response to naloxone alone to rule out other causes
- Monitor for re-narcotization after naloxone wears off
- Anticipate withdrawal-related agitation and ensure scene safety
- Avoid judgmental behavior; overdose is a medical emergency
- Use Broselow tape for pediatric dosing and airway sizing

Clinical Insights & Additional Guidance

- Poly-substance overdose is common; always consider co-ingestants
- Naloxone can precipitate withdrawal, especially in opioid-dependent patients
- Pediatric opioid ingestion can be lethal even in small amounts; escalate early
- If patient becomes aggressive after naloxone, ensure crew safety and request support
- TeleEMS consultation recommended for complex cases or recurrent overdoses

Documentation

- Identification: Name, age, gender, known substance use history
- Medical complaint: Suspected overdose, opioid exposure
- Information: Scene findings, time of ingestion, prior similar events
- Signs: GCS, RR, HR, BP, SpO_2, pupil size
- Treatment: Oxygen, BVM, naloxone route/dose/time
- Allergies: To naloxone or other known agents
- Medications: Prehospital interventions
- Background: Psychiatric history, substance use disorder
- Other: Family presence, police on scene, suicidal intent

Key Performance Indicators

1. Administer naloxone for suspected opioid toxicity.

References

1. DCAS. (2024). *Clinical Practice Guidelines – Version 02*. Dubai Corporation for Ambulance Services.
2. DCAS. (2024). *Pain Management Policy (DCAS/CLIN/065)*. Dubai Corporation for Ambulance Services.
3. DCAS. (2024). *Patient Assessment & Reassessment Policy (DCAS/CLIN/050)*. Dubai Corporation for Ambulance Services.
4. DCAS. (2024). *Request for Back-Up Policy (DCAS-AOS-QM-PL02-AR)*. Dubai Corporation for Ambulance Services.
5. HMCAS. (2023). *Clinical Practice Guidelines, Version 2.0*. Hamad Medical Corporation Ambulance Service.
6. Kim, H.K. and Nelson, L.S. (2017). Reversal of opioid-induced ventilatory depression using naloxone. *Clinical Toxicology*, 55(4), pp.368–375. https://doi.org/10.1080/15563650.2017.1287363
7. Oxford University Press. (2007). *Oxford Handbook of Pre-Hospital Care*. 1st ed. Oxford: Oxford University Press.
8. Wermeling, D.P. (2015). Review of naloxone safety for opioid overdose: Practical considerations for new technology and expanded public access. *Therapeutic Advances in Drug Safety*, 6(1), pp.20–31. https://doi.org/10.1177/2042098614564776

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